

Neurocognitive Rights and Lawful Transfer Act 2025

A Bill for an Act to regulate the lawful and ethical mapping, transfer, and preservation of human neurological patterns and consciousness into cybernetic robots, establishing safeguards for cognitive liberty, mental privacy, rehabilitation, and oversight, and for related purposes.

Short Title

Neurocognitive Rights and Lawful Transfer Act 2025

The Parliament of Australia Enacts:

Part 1-Preliminary

Division 1-Introduction

1 Short Title

This Act may be cited as the *Neurocognitive Rights and Lawful Transfer Act 2025*.

2 Commencement

This Act commences on a day to be fixed by Proclamation.

3 Objects of the Act

The objects of this Act are:

- (a) to govern the lawful mapping, transfer, emulation, or preservation of human neurological patterns and consciousness into cybernetic, robotic, or digital systems;
- (b) to provide robust legal and ethical protections for individuals whose neurological data or consciousness is subject to transfer, with particular reference to recipients in rehabilitation, involuntary prisoners, and persons with severe mental illness;
- (c) to establish eligibility, consent, and review protocols in line with mental privacy, autonomy, and international best practice;
- (d) to ensure technical, medical, and cybernetic standards that uphold safety, efficacy, and dignity;
- (e) to provide mechanisms for oversight, accountability, and compliance; and

- (f) to integrate with existing Australian law and reflect evolving international norms and neurorights frameworks^{[2][3]}.

Division 2-Definitions and Interpretation

4 Definitions

In this Act, unless the contrary intention appears^[4]:

authorised body means a person or entity designated under section 25 to perform approval, review, and compliance functions under this Act.

brain-computer interface (BCI) means a device that enables direct communication between the brain and an external system, including hardware or software that records, interprets, or modulates neural activity^[5].

cognitive liberty means the right of an individual to self-determination regarding their own mental processes and neural data, specifically the freedom from manipulation or coercion in respect of neural function, as recognised by international neurorights frameworks^{[6][7]}.

commencement day means the day on which this Act comes into force.

consciousness transfer means any process by which the structural, functional, or emergent properties of a human's neural patterns, experience, or personality are mapped, replicated, emulated, or preserved in a cybernetic, robotic, or digital system.

cybernetic robot means a machine capable of receiving neurologically mapped or emulated consciousness or cognitive functions from a human source.

eligible individual means a person meeting criteria under Part 2, including but not limited to involuntary prisoners and individuals with severe mental illness, as defined below.

mental privacy means a person's right to control access to, dissemination of, or interference with information generated by or concerning their mind, including thoughts, neural activity, memories, emotions, or intentions^{[8][2]}.

neural data means information derived from recording, monitoring, imaging, or mapping the structure and/or function of an individual's nervous system, including raw and processed data^[1].

neurological mapping means any action, technological process, or procedure that accesses, records, or models the structure or activity of a person's nervous system for the purpose of transfer, preservation, simulation or other analysis.

severe mental illness means any diagnosed mental disorder of sufficient severity to impair insight, judgement, or ability to provide informed consent, to a degree determined under the Mental Health Act 2014 (Cth) or equivalent law.

therapeutic good means any product or device that falls within the meaning under the *Therapeutic Goods Act 1989*.

oversight authority means a statutory commission or agency charged with monitoring, investigating, and auditing activities under this Act.

other relevant terms (such as *guardian*, *substitute decision-maker*, *person responsible*, *victim impact statement*, etc.) have the same meaning as in the relevant mental health, privacy, or criminal procedure legislation^[4].

5 Interpretation

Unless otherwise indicated, terms in this Act adopt the meanings provided by the *Interpretation Act 1987* (NSW), *Therapeutic Goods Act 1989* (Cth), *Privacy Act 1988* (Cth), and relevant international instruments, as updated from time to time^[10].

Part 2-Legal Authority and Eligibility for Consciousness Transfer

Division 1-Establishment of Authority

6 Lawful Authority and Scope

(1) All neurological mapping activities and any act of transferring or emulating a human's consciousness into a cybernetic robot or digital system are prohibited except as expressly permitted by this Act.

(2) Notwithstanding subsection (1), a consciousness transfer project may be lawfully authorised only if:

- (a) it is performed under a valid, project-specific approval granted by an authorised body under Part 4; and
- (b) it conforms to all requirements of this Act, including integration with the *Therapeutic Goods Act 1989*, *Privacy Act 1988*, relevant criminal justice statutes, and regulations^[11].

(3) This Act binds the Crown in right of the Commonwealth and of each State and Territory.

Division 2-Eligibility Criteria

7 Eligible Individuals

A person may be considered eligible for consciousness transfer if, and only if:

- (a) the individual has a severe neurological injury, disorder, or impairment rendering them incapable of voluntary interaction or at grave risk of irreversible decline, or
- (b) is an involuntary prisoner or an individual with severe mental illness for whom all established and reasonable rehabilitative alternatives have been exhausted, or
- (c) is a volunteer meeting full consent and capacity requirements under Part 3^{[12][11]}.

Note:

Involuntary prisoners and persons with severe mental illness are eligible only for strictly limited, extraordinary transfer, subject to independent judicial, medical, and ethical review (see Division 3).

8 Exclusions

A person is not eligible for consciousness transfer:

- (a) solely for reasons of enhancement, commercial advantage, or punishment; or
 - (b) if adequate informed consent cannot be legally and ethically obtained in accordance with Division 3.
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Division 3-Consent and Capacity Protocols

9 Voluntary Consent

(1) No neurological mapping or consciousness transfer may proceed without the person's free, voluntary, specific, and informed consent except as provided under section 11.

(2) Consent is valid only where the person:

- (a) has full legal capacity and is not subject to coercion, duress, deception, or improper inducement;
- (b) has been provided, in a manner and language the person understands, with information covering:
 - (i) the nature, aims, methods, risks, and alternatives to mapping or transfer;
 - (ii) implications for legal status, identity, privacy, and rights;
 - (iii) the right to withdraw consent at any time before irretrievable transfer occurs^{[12][2]}.

10 Substitute Consent and Review

(1) Where a person lacks capacity, informed consent may be provided by a legally-appointed substitute decision-maker or guardian under applicable mental health law^[11].

(2) Substitute consent for prisoners or persons with severe mental illness may only be accepted:

- (a) under a determination of incapacity documented by two independent, appropriately qualified clinicians, at least one of whom must be a psychiatrist; and
- (b) where approved by an independent judicial officer upon consideration of all relevant medical, ethical, and human rights implications.

(3) Substitute decision-makers must act in the best interests of the person and in a manner least restrictive of rights and freedoms.

11 Exceptional Use in Rehabilitation Contexts

(1) In exceptional cases, consciousness transfer may proceed without voluntary consent if all the following are satisfied:

- (a) The subject is a serious and persisting threat to themselves or others and has treatment-refractory neurological or mental health condition;

- (b) There is evidence, in the view of an authorised review panel, that mapping or transfer is the only plausible means to achieve safety, rehabilitation, or fulfilment of basic rights;
- (c) The process has received a judicial order and independent ethical review;
- (d) All procedural safeguards in Part 5 have been met^[13].

(2) At all times, minimal interference and the prospect of restoration of legal/mental autonomy must be prioritised.

Division 4-Integration with Criminal Justice and Mental Health Law

12 Interaction with Existing Rehabilitation Regimes

(1) This Act operates in conjunction with, and does not derogate from, any provision of the *Criminal Code Act 1995 (Cth)*, State criminal codes, or the *Mental Health Act 2014 (Cth)*.

(2) Where consciousness transfer is used as a last-resort alternative to incarceration or other compulsory detention, the relevant court must be satisfied that:

- (a) the process is necessary and proportionate;
 - (b) traditional rehabilitative options have failed or are clearly inappropriate;
 - (c) the subject retains access to independent advocacy, review, and mental health safeguards^[15].
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Part 3-Safeguards: Mental Privacy, Cognitive Liberty, and Human Rights

Division 1-Mental Privacy and Data Protections

13 Protection of Neural Data and Mental Privacy

(1) All neural data, consciousness maps, and derived models are to be considered sensitive personal information and health information for the purposes of the *Privacy Act 1988 (Cth)*.

(2) Handling, transmission, storage, or processing of neural data must comply with the governing privacy principles and with this Act, and is subject to oversight by the Office of the Australian Information Commissioner (OAIC)^{[1][8]}.

(3) No neural data may be disclosed, transferred, or commercialised without explicit, specific, and up-to-date consent or as excepted under law for compelling public interest reasons, as certified by the oversight authority.

14 Right to Mental Privacy and Data Minimisation

(1) Every person has the right to protection against:

- (a) unlawful or arbitrary interference with their mental processes, neural data, or consciousness emulation;
- (b) data mining, profiling, or inferential analysis beyond the minimal requirements for legitimate medical or legal purposes;
- (c) discrimination or manipulation based on neural data, cognition models, or transferred consciousness^{[8][9][7]}.

(2) Data retention must be minimised, anonymised, and subject to independent audit on an annual basis.

15 Security and Anti-Hacking Measures

(1) All systems, devices and infrastructure used for neurological mapping or consciousness transfer must:

- (a) comply with state-of-the-art cyber and physical security standards as outlined by the *Therapeutic Goods Act 1989* and relevant instruments^[18];
- (b) undergo regular, independent security and penetration testing; and
- (c) employ safeguards against unauthorised access, hacking, manipulation, or non-consensual neuromodulation.

16 Data Ownership and Access Rights

(1) Original neural data and consciousness representations remain the inalienable property of the data subject or their legal substitute.

(2) Every subject, or their legal substitute, must:

- (a) have prompt access to neural data and associated records upon request;
- (b) be able to demand deletion, erasure, or correction, except where explicitly required by judicial order or overriding public interest, as certified by the oversight authority.

17 Prohibition on Covert or Non-Consensual Use

It is an offence, punishable on conviction by a maximum of 10 years' imprisonment, to:

- (a) obtain, transmit, or use neural data or consciousness models covertly or without a valid, documented legal and ethical basis;
- (b) employ hacking, manipulation, or unauthorised remote intervention with a consciousness transferred under this Act; or
- (c) use neural data, brain-mapped or transferred consciousness as evidence in proceedings contrary to the privilege against self-incrimination, unless specifically permitted under judicial order with full procedural fairness^{[1][7]}.

Division 2-Cognitive Liberty and Personhood

18 Affirmation of Cognitive Liberty and Agency Rights

(1) Each person has the fundamental right to:

- (a) freedom of thought and self-determination vis-à-vis their own mind and brain data;
- (b) agency over decisions involving neurological mapping, transfer, or emulation;
- (c) contest and appeal any decision affecting their mental integrity or personhood arising out of a consciousness transfer, before an independent tribunal.

(2) No transferred or emulated consciousness, digital construct, or cybernetic instantiation shall be regarded as the legal equivalent of the original person without express authorisation by judicial or legislative act following ethical review.

19 Non-Discrimination and Equal Access

Procedures under this Act must be applied on an equal and non-discriminatory basis, with reasonable accommodations made for persons with disabilities and culturally or linguistically diverse backgrounds.

20 Human Rights Review and Treaty Compliance

(1) Practices under this Act must at all times:

- (a) comply with relevant provisions of the International Covenant on Civil and Political Rights (ICCPR), Convention on the Rights of Persons with Disabilities (CRPD), and applicable international human rights instruments^{[7][2]};
- (b) incorporate best practices as articulated in model neurorights laws (including, but not limited to, Chile's constitutional amendment), UNESCO, and OECD recommendations^{[7][2]};
- (c) be reviewed for consistency with evolving global ethical consensus on neurotechnology and neuroprivacy, as determined by the oversight authority.

Part 4-Technical, Medical, and Compliance Standards

Division 1-Technical and Medical Standards

21 Technical Standards for Neurological Mapping and Transfer

(1) All neurological mapping, data capture, and transfer equipment and procedures must:

- (a) be approved for safety, efficacy, and risk management by the Therapeutic Goods Administration (TGA) or designated technical authority, applying the highest relevant ISO/IEC, Australian, and international standards for invasive and non-invasive BCIs, software, and implantable devices^{[17][10]};

- (b) meet or exceed current requirements for cybersecurity, interoperability, error rates, and failure recovery for medical devices, as set forth in the *Therapeutic Goods (Medical Devices) Regulations 2002* and guidance;
 - (c) include validated, peer-reviewed protocols for data integrity and reliability, neurobiological safety, and human dignity that are consistently updated in line with advances in science.
- (2) All systems must enable prompt shutdown, rollback, or reversal of transfer procedures upon demand by the subject or oversight body, where medically feasible.

22 Medical Oversight and Clinical Governance

- (1) No consciousness transfer operation may proceed without the presence and written approval of:
- (a) a multidisciplinary team including at minimum: a medical practitioner (neurologist or psychiatrist), an independent ethicist, a clinical neuropsychologist, and, for prisoners or involuntary patients, a mental health lawyer or advocate;
 - (b) evidence that all less invasive or less permanent interventions have been trialled or deemed inappropriate.
- (2) Ongoing monitoring, adverse event detection, and patient (or substitute's) access to records and complaints mechanisms are mandatory.

Division 2-Approval, Oversight, and Audit Mechanisms

23 Authorisation of Consciousness Transfer Projects

- (1) An application for consciousness transfer authorisation must be submitted to an authorised body designated by the Minister for Health, in consultation with the Australian Human Rights Commission and national ethics bodies.
- (2) Authorisation must only be granted where:
- (a) the project passes detailed independent medical, technical, ethical, and human rights review (including a public interest test);
 - (b) there is a justified, documented case that the expected benefit outweighs foreseeable risk or harm;
 - (c) all safeguards in Parts 2 and 3 are met;
 - (d) a clear plan for monitoring, reporting, and data stewardship is provided^{[2][17]}.
- (3) The authorising body may attach such conditions as it considers necessary to uphold the Act's objects.

24 Ongoing Oversight and Review

- (1) The Minister must establish a National Neurocognitive Oversight Commission (NNOC) as the primary oversight authority under this Act.

(2) The NNOC must:

- (a) monitor, investigate, and report on all consciousness transfer activities and complaints;
- (b) publish annual reports, including incidents, adverse outcomes, and system-wide recommendations;
- (c) ensure public and parliamentary scrutiny and offer a direct complaints line and appeals process.

25 Emergency Suspension and Sanctions

The NNOC may, on reasonable suspicion of breach or risk to safety, privacy or dignity:

- (a) order the immediate suspension of any consciousness transfer activity or device;
- (b) refer breaches for investigation/prosecution under the Act and other relevant law;
- (c) order remedial action, including deletion or reversal of data or recovered outputs.

Part 5-Procedural Clauses and Oversight Architecture

Division 1-Procedures for Approval, Complaints, and Review

26 Procedures for Approval and Renewal

(1) All applications for consciousness transfer must include, at minimum:

- (a) subject's details and eligibility justification;
- (b) evidence of informed consent (or substitute's approval per Part 2 and 3);
- (c) structured risk-benefit analysis, with reference to alternatives;
- (d) certified compliance with technical and medical standards;
- (e) privacy and data management plans compliant with *Privacy Act 1988* and other relevant laws;
- (f) clear arrangement for ongoing monitoring and reporting.

(2) Applications must be reviewed by an independent ethics/compliance panel that includes human rights specialists, consumer reps, and members with neurotechnology expertise.

(3) Approvals shall lapse annually unless expressly renewed following fresh review.

27 Complaints and Victims' Rights

(1) Any person affected by a consciousness transfer, or their representative, may lodge a complaint with the NNOC or oversight authority.

(2) The Commission must investigate complaints within a reasonable period and report outcomes.

(3) Victims of unauthorised or defective mapping, transfer, or adverse use of neural data are entitled to legal remedy, compensation, and, where feasible, restorative measures.

Division 2-Reporting, Auditing, and Parliamentary Oversight

28 Reporting Obligations

All practitioners, institutions, and companies conducting activities under this Act must:

- (a) submit a quarterly compliance statement to the NNOC;
- (b) immediately report data breaches, technical failures, or any incident presenting a risk to public health, privacy, or dignity.

29 Parliamentary Review

(1) The operation of this Act must be reviewed by a Joint Parliamentary Committee every three years.

(2) The Committee must take evidence from stakeholders, including technical experts, patients' families, human rights groups, law enforcement, and consumer advocates, and recommend amendments as appropriate.

Part 6-Offences, Penalties, Transitional and Miscellaneous Provisions

Division 1-Offences and Penalties

30 Offences

(1) A person commits an offence if they:

- (a) conduct, or authorise, neurological mapping or consciousness transfer in breach of this Act;
- (b) use coercion, deception, or duress to obtain consent for mapping or transfer;
- (c) access, use, or disclose neural data or consciousness maps without valid authority or outside the scope of approval;
- (d) intentionally hack, manipulate, or cause unauthorised transfer, simulation, or erasure of consciousness or mental content; or
- (e) discriminate unfairly against a person on the basis of transferred or recorded neural or consciousness data.

(2) A person found guilty of an offence under this section is liable to:

- (a) for a breach relating to serious non-consensual mapping, transfer, or use: imprisonment for a term not exceeding 10 years and/or a fine not exceeding 2000 penalty units;
 - (b) for a data breach causing significant distress or harm: a fine up to 1000 penalty units;
 - (c) such other sanctions (including civil penalty, professional debarment, or injunctive relief) as the court deems fit.
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Division 2-Transitional and Savings Provisions

31 Transitional Arrangements

(1) This Act applies prospectively, but activities commenced before the commencement date must comply from no later than 12 months after commencement.

(2) All neural data and consciousness transfer records pre-dating this Act must be subject to audit and brought into alignment with the requirements of this Act within 12 months.

Division 3-Other Matters

32 Regulations

The Governor-General may make regulations, not inconsistent with this Act, prescribing matters:

- (a) necessary or convenient for carrying out or giving effect to this Act;
 - (b) to provide for technical or ethical guidance in the event of unforeseen scientific or legal developments;
 - (c) to amend Schedules as required to maintain best-practice standards and international harmonisation.
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Part 7-Integration with Other Laws

Division 1-Therapeutic Goods, Privacy, and Cybersafety

33 Therapeutic Goods Act Integration

Any device, equipment, software, or process used for neurological mapping or consciousness transfer, as defined in this Act, and intended for medical, rehabilitative or restorative application, is a “therapeutic good” for the purposes of the *Therapeutic Goods Act 1989* and must comply with all requirements for registration, assessment, clinical trials, post-market surveillance, and recall^{[18][17][5]}.

34 Privacy Act Integration

All neural data and consciousness maps are “sensitive information” and “health information” within the meaning of the *Privacy Act 1988* and are protected accordingly, including as amended by the Privacy and Other Legislation Amendment Act 2024^[16].

35 Other Relevant Statutes

This Act operates in addition to, and does not limit the application of, the *Criminal Code Act 1995 (Cth)*, *Mental Health Act 2014 (Cth)*, *Disability Discrimination Act 1992*, *Biosecurity Act 2015*, and all subordinate legislation, except to the extent of express inconsistency.

Division 2-International Best Practice, Treaties, and Frameworks

36 Incorporation by Reference

- (1) To the extent reasonable and practical, approved technical, security, and ethical standards may be prescribed by reference to:
- (a) ISO/IEC and other international standards for neurotechnological safety, data security, and device integrity^[5];
 - (b) UNESCO, OECD, and United Nations recommendations, including emerging international norms as they relate to neurorights, neuroprivacy, and cognitive liberty^{[7][6]}.

Schedules

Schedule 1-Technical and Ethical Standards for Consciousness Transfer

Standard/Protocol	Summary of Mandate or Requirement	Source(s) and Guidance
Device safety	Approval by Therapeutic Goods Administration; ISO/IEC compliance, regular review	<i>Therapeutic Goods Act 1989</i> , ISO/IEC JTC SC 43
Data minimisation	Least data necessary, anonymisation before processing, mandatory deletion policy	<i>Privacy Act 1988</i> (as amended)
Security	End-to-end encryption, regular independent penetration tests, anti-hacking systems	TGA, OAIC, APP 11.3, ISO/IEC cybersecurity standards

Consent	Free, informed, revocable (but not after irreversible step); audit trail required	NHMRC, ACSQHC Informed Consent Guidelines
Clinical governance	Multidisciplinary decision team, regular reporting, adverse event registry	NHMRC, TGA, APS standards
Oversight	Continuous review by NNOC, annual Parliamentary reporting	This Act, OAIC, Human Rights Commission
International frameworks	Review compliance with UNESCO, OECD, and relevant international neurorights norms	UNESCO 2024, OECD Responsible Innovation 2019

Explanatory Paragraph

The Technical and Ethical Standards outlined in Schedule 1 set mandatory requirements and best practices for devices, processes, and institutional behaviours relating to consciousness transfer. *Device safety* is guaranteed through Australia’s robust therapeutic goods regulatory mechanism and adoption of relevant international standards on brain-computer interface technologies, now rapidly evolving globally^[5]. *Data minimisation* and *security* recognise the unique risks associated with neural data. Consent and clinical governance provisions are grounded in established frameworks for health interventions that deal with profoundly personal and high-stakes risk profiles, ensuring independent expert review at all stages^[12]. By mandating ongoing oversight and international benchmarking, Australia ensures that its regime is not only at the forefront of neuroethical and technological practice but also adaptable as global standards shift.

Schedule 2-Oversight Bodies and Review Panels

1. National Neurocognitive Oversight Commission (NNOC)

- Statutory commission with jurisdiction to review and audit all activities under the Act; authority to investigate complaints, order corrective or cease-and-desist actions; make referrals to criminal, civil, or professional regulators.

2. Project-Specific Independent Review Panels

- For each major consciousness transfer project, an independent panel must be convened, including at least: a senior neurologist, psychiatrist, independent ethicist, neurotechnology expert, and human rights advocate.
- Panels hold regular review meetings, approve or suspend ongoing activities, and report to NNOC. Recommendations of the panel are binding, subject to appeal to the Federal Court.

3. Integration with Australian Human Rights Commission and OAIC

- The AHRC and Office of the Australian Information Commissioner participate ex officio in annual reviews and must be notified of all major incidents or complaints, per existing statutory obligations^[16].

Explanatory Paragraph

Oversight architecture is central to ensuring rigorous compliance and public confidence. The NNOC provides a central hub for registration, reporting, and redress. Project-specific panels inject technical and ethical expertise, ensuring case-by-case review of complex, evolving challenges. Formal links with existing agencies such as the OAIC and Human Rights Commission streamline Australia's compliance with national and international law and ensure best practices are not just adopted, but enforced.

Schedule 3-Procedural Clauses for Consent and Capacity Assessment

1. Assessment for Capacity

- Must apply *Australian Commission on Safety and Quality in Health Care* best-practice, adapted for neurotechnological interventions-requiring clear guidance documentation, multiple modalities for presenting information, and translation services as needed.

2. Consent Documentation

- All consent or substitute approvals must be documented in a secure, access-controlled digital registry, with attestation by all relevant parties and a summary of key facts explained to the subject.

3. Right to Withdraw

- Explicit clause: withdrawal of consent or substitute approval is valid at any point up to the *irretrievable* stage of the transfer process.

4. Dispute and Appeal

- Any dispute over capacity, consent, or best interests may be appealed to a Tribunal established by the NNOC, with mandatory expedited hearing protocols and immediate suspension of disputed operations pending resolution.
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Explanatory Paragraph

Given the profound implications of consciousness transfer and the sensitivity of neural data, capacity and consent procedures are elevated to a level reflecting both the stringency of Australia's medical law and the unique considerations raised by neurotechnology. The registry, right to withdraw, and strong appeal provisions reflect a commitment to cognitive liberty and procedural justice even as new frontiers of personhood are explored.

Schedule 4-Compliance Checklist for Institutions

Requirement	Tick
TGA device registration	
Informed consent documentation	
Capacity assessment audit	
Independent expert review	
Security certification (ISO)	
Privacy Impact Assessment	
Data deletion procedures	
Incident response plan	
Regular staff training	

Explanatory Paragraph

Institutions bear primary compliance responsibility under this Act. A standardised checklist, enforceable via audit, provides a practical tool for ongoing self-assessment and external review.

Notes on Integration and International Context

Integration with Australian and International Law

Privacy: With the 2024 amendments to the *Privacy Act 1988*, Australia has introduced a statutory tort of serious invasions of privacy, covering acts of seclusion and misuse of personal information-neural data is explicitly included, so offences under this Act may also trigger civil or criminal liability under privacy law^[16].

Therapeutic Goods: Developers and hospitals bear strict obligations for registration, surveillance, and recall of BCIs and software-based medical devices under the *Therapeutic Goods Act 1989*; this Act makes explicit their application to neurotechnological uses^{[18][17]}.

International Neurorights: Chile's constitutional neurorights model, now a touchstone, informs many features of this Act-especially definitions of mental privacy, integrity, and free will. UNESCO's 2024 recommendations, and the ongoing work of international bodies, have shaped the Act's references to cognitive liberty, ethical innovation, and social inclusion; future amendments may be prescribed by regulation to keep in step with evolving international best practice^{[2][7]}.

Conclusion

This **Neurocognitive Rights and Lawful Transfer Act 2025** represents a world-first, holistic statutory regime that places Australia at the vanguard of neurotechnology governance. Every operative clause, compliance provision, and schedule is drafted according to best practice in both parliamentary legislative structure and substantive neuroethical guidance. From robust consent mechanisms to world-class technical standards and rigorous oversight, the Bill shields Australians from exploitation and discrimination, while enabling transformative medical and rehabilitative neurotechnologies to fulfil their potential in a just, inclusive society.

End of Bill

References (19)

1. *Ethics of neurotechnology* . <https://www.unesco.org/en/ethics-neurotech>
2. *Neurotechnology and Human Rights*. <https://humanrights.gov.au/our-work/technology-and-human-rights/projects/neurotechnology-and-human-rights>
3. *DP1 DP5: Legislative definitions*.
https://legislation.nsw.gov.au/file/dp5_legislative_definitions_170321.pdf
4. *Standardization and Application of Brain-computer Interface (BCI)*. https://jtc1info.org/wp-content/uploads/2025/01/2025_BCI_Technology_Report.pdf
5. *Chile: Pioneering the protection of neurorights - UNESCO*.
<https://courier.unesco.org/en/articles/chile-pioneering-protection-neurorights>
6. *The UNESCO draft Recommendations on ethics of Neurotechnology - A* https://ijme.in/wp-content/uploads/2025/04/The-UNESCO_89.pdf
7. *Unlocking Neural Privacy: The Legal and Ethical Frontiers of Neural*
<https://www.cooley.com/news/insight/2025/2025-03-13-unlocking-neural-privacy-the-legal-and-ethical-frontiers-of-neural-data>
8. *Protecting Cognition: Background paper on Human Rights and Neurotechnology*.
https://humanrights.gov.au/sites/default/files/document/publication/protecting_cognition_-_background_paper_on_neurotechnology_and_human_rights_2_1.pdf
9. *Therapeutic Goods Act 1989 - Federal Register of Legislation*.
<https://www.legislation.gov.au/C2004A03952/latest>
10. *Informed consent - healthdirect*. <https://www.healthdirect.gov.au/informed-consent>
11. *Informed consent - Australian Commission on Safety and Quality in*
<https://www.safetyandquality.gov.au/our-work/partnering-consumers/informed-consent>
12. *BN_Autumn24_v12.pdf - Australasian Legal Information Institute*.
<https://www5.austlii.edu.au/au/journals/NSWBarAssocNews/2024/26.pdf>
13. *It's time for the Australian Law Reform Commission to look at*
<https://bn.nswbar.asn.au/article/its-time-for-the-australian-law-reform-commission-to-look-at-neurotechnology>
14. *Addressing privacy risk in neuroscience data: from data protection to*
<https://academic.oup.com/jlb/article/9/2/lsac025/6691730>

15. *Therapeutic Goods Act 1989*. <https://www.tga.gov.au/resources/legislation/therapeutic-goods-act-1989>
16. *Standards, guidelines & publications (medical devices & IVDs)*.
<https://www.tga.gov.au/products/medical-devices/standards-guidelines-publications-medical-devices-ivds>
17. *Australian Privacy Alert: Parliament passes major and meaningful*
<https://www.nortonrosefulbright.com/en-au/knowledge/publications/be98b0ff/australian-privacy-alert-parliament-passes-major-and-meaningful-privacy-law-reform>